

26SMCV01149 26SMCV01149

County: los-angeles

Division: Civil Law and Motion

Department: 205

Hearing date: 2026-09-16

Source URL: https://www.lacourt.ca.gov/tentativeRulingNet/ui/main.aspx?casetype=civil&dept_date=BH%20%2C205%2C09%2F16%2F2026

Source SHA-256: bfe32f05c588b0f5644f7b16f85a6e31dc3f5865ab74b10915815b8a4c6e036d

Case Number: 26SMCV01149 Hearing Date: September 16, 2026 Dept: 205

Superior Court of California

County of Los Angeles – West District

Beverly Hills Courthouse / Department 205

KATHLEEN JENSEN,

Plaintiff,

v.

CEDARS SINAI MEDICAL CENTER, et al.,

Defendants.

Case No.: 26SMCV01149

Hearing Date: September 16, 2026

[TENTATIVE] order RE:

defendants cedars sinai marina

hospital and timothy diperi

m.d.'s DEMURRER TO and motion

to strike first amended

COMPLAINT

BACKGROUND

This is a medical malpractice and elder abuse case. On March 26, Plaintiff Kathleen Jensen was seen in the emergency room at Defendant Cedars Sinai Marina del Rey Hospital ("Cedars") for abdominal pain. (First Amended Complaint ("FAC"), ¶ 9.) She was then admitted to the hospital the following day, where she remained for an extended period, until at least early April.

Throughout her stay, Plaintiff was seen by various doctors, who she claims failed to properly diagnose a small bowel obstruction, which led to a "pneumoperitoneum with two areas of colonic perforation and fecal peritonitis." (Id. ¶ 74.) As a result of each of their failures, Plaintiff has required multiple surgeries, suffered repeated in-patient hospital stays, and incurred bills from hospital. (Id. ¶¶ 80-81.)

As relevant to this hearing, Plaintiff alleges that she was seen twice by Dr. Timothy Diperi, a surgeon. Dr. Diperi saw her on March 28, where he noted that the CT scan of her abdomen showed "scattered colonic diverticula. No evidence of diverticulitis. Small amount of free fluid in pelvis". (Id. ¶ 28.) Dr. Diperi conducted a physical examination which showed the "abdomen mildly distended, mildly tender to palpation in lower abdomen". (Id. ¶ 31.) He recorded no bowel sounds and did not do a pelvic-rectal examination. (Id.) Dr. Diperi (along with another doctor) concluded that there was no evidence of diverticulitis and no acute surgical intervention was needed at this time. The two doctors recommended aggressive bowel regimen and "please call back surgery with additional questions or concerns". (Id. ¶ 33.)

Dr. Diperi saw Plaintiff again a second time, on April 1, 2025. At that exam, Plaintiff claims Dr. Diperi failed to notice that Plaintiff was tachycardic on March 31, and that her respiratory rate was elevated. He failed to note that there had been a 12-pound weight gain since admission. (Id. ¶ 48.) Dr. DiPeri stated that the abdomen was "moderately distended, diffuse mild tenderness to palpation." His notes do not indicate whether he examined Ms. Jensen for guarding or rebound, and he did not record any bowel sounds. (Id. ¶ 49.)

Three days later, on April 4, Plaintiff underwent exploratory surgery, and she was found to have 2 areas of colonic perforation and fecal peritonitis. (Id. ¶ 62.) Plaintiff continued to deteriorate after the first operation and was returned to the operating room on April 5, 2025, where she had a small bowel resection for ischemic bowel and wound vac placement. (Id. ¶ 63.)

This action ensued. Plaintiff sued her doctors as well as Defendant Cedars Sinai Marina Hospital, alleging three claims for (1) medical malpractice, (2) elder abuse and (3) vicarious liability.

This hearing is on Defendants Cedars Sinai Marina Hospital and Dr. Diperi's demurrer to the second and third causes of action and motion to strike portions of the FAC. Defendants argue that Plaintiff's elder abuse claim is uncertain and ambiguous; the FAC alleges at most professional negligence, not elder abuse, and Plaintiff's vicarious liability claim is not a standalone cause of action but a theory of derivative liability. Defendants also move to strike Plaintiff's prayer for punitive or exemplary damages on the ground that the allegations do not support a finding of fraud, malice or oppression. Additionally, Defendants move to strike Plaintiff's request for attorneys' fees because there is no statutory or contractual basis to award Plaintiff such fees.

MEET AND CONFER

Code Civ. Proc. §§ 430.41 and 435.5 requires that before the filing of a demurrer or a motion to strike, the moving party "shall meet and confer in person or by telephone" with the party who filed the pleading that is subject to demurrer or a motion to

strike. Moving Defendants submit the Declaration of Allen William II who fails to attest that the parties met and conferred by telephone or in person. Notwithstanding, the Court cannot overrule a demurrer or deny a motion to strike based on an insufficient meet and confer.

LEGAL STANDARD

"[A] demurrer tests the legal sufficiency of the allegations in a complaint." (Lewis v. Safeway, Inc. (2015) 235 Cal.App.4th 385, 388.) A demurrer can be used only to challenge defects that appear on the face of the pleading under attack or from matters outside the pleading that are judicially noticeable. (See Donabedian v. Mercury Ins. Co. (2004) 116 Cal.App.4th 968, 994 (in ruling on a demurrer, a court may not consider declarations, matters not subject to judicial notice, or documents not accepted for the truth of their contents).) For purposes of ruling on a demurrer, all facts pleaded in a complaint are assumed to be true, but the reviewing court does not assume the truth of conclusions of law. (Aubry v. Tri-City Hosp. Dist. (1992) 2 Cal.4th 962, 967.)

Further, the court may, upon motion, or at any time in its discretion, and upon terms it deems proper, strike any irrelevant, false, or improper matter inserted in any pleading. (Code Civ. Proc., § 436, subd. (a).) The court may also strike all or any part of any pleading not drawn or filed in conformity with the laws of this state, a court rule, or an order of the court. (Code Civ. Proc., § 436, subd. (b).) The grounds for a motion to strike are that the pleading has irrelevant, false, or improper matter, or has not been drawn or filed in conformity with laws. (Code Civ. Proc., § 436.) The grounds for moving to strike must appear on the face of the pleading or by way of judicial notice. (Code Civ. Proc., § 437.)

Leave to amend must be allowed where there is a reasonable possibility of successful amendment. (See Goodman v. Kennedy (1976) 18 Cal.3d 335, 349 (court shall not "sustain a demurrer without leave to amend if there is any reasonable possibility that the defect can be cured by amendment"); Kong v. City of Hawaiian Gardens Redevelopment Agency (2002) 108 Cal.App.4th 1028, 1037 ("A demurrer should not be sustained without leave to amend if the complaint, liberally construed, can state a cause of action under any theory or if there is a reasonable possibility the defect can be cured by amendment."). The burden is on the complainant to show the Court that a pleading can be amended successfully. (Blank v. Kirwan (1985) 39 Cal.3d 311, 318.)

DISCUSSION

Elder Abuse

Moving Defendants argue that Plaintiff's elder abuse claim is uncertain. The Court disagrees.

Demurrers for uncertainty under Code of Civil Procedure section 430.10, subdivision (e) are disfavored. (Lickiss v. Financial Industry Regulatory Authority (2012) 208 Cal.App.4th 1125, 1135.) "A demurrer for uncertainty is strictly construed, even where a complaint is in some respects uncertain, because ambiguities can be clarified under modern discovery procedures." (Khoury v. Maly's of California, Inc. (1993) 14 Cal.App.4th 612, 616.)

Generally, a demurrer for uncertainty is reserved for instances where the pleading is so incomprehensible that a defendant cannot reasonably respond. That is not the case here. Defendants are clearly on notice that Plaintiff is suing for the care (or lack of care) he received for his abdominal pain while he was being treated by doctors at Cedars from March 26 to April 4.

Moving Defendants next argue that Plaintiff's claim for elder abuse fails because Plaintiff has not alleged neglect or the withholding of care, but rather substandard medical care. The Court agrees.

In *Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, the California Supreme Court explained that, "[a]s used in the Act, neglect refers not to the substandard performance of medical services but, rather, to the 'failure of those

responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.’ [Citation.] Thus, the statutory definition of ‘neglect’ speaks not of the undertaking of medical services, but of the failure to provide medical care.” (Id. at 783.)

In *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, the California Court of Appeal, distilled from the Act and case law, including *Covenant Care*, “several factors that must be present for conduct to constitute neglect within the meaning of the Elder Abuse Act.” (Id. at 406.) These requirements include that the plaintiff must allege, and ultimately prove by clear and convincing evidence, that the defendant “denied or withheld goods or services necessary to meet the elder or dependent adult’s basic needs, ... with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness).” (Id. at 406–407.)

The court concluded that the plaintiffs had failed to adequately allege elder abuse where they “allege[d] that [the decedent] died because the Hospital did not administer the antibiotics [the decedent] needed to treat his pneumonia and did not have the proper size endotracheal tube in the crash cart.” (Id. at 408.) The court observed that “[t]hese allegations indicate the Hospital did not deny services to or withhold treatment from [the decedent]—on the contrary, the staff actively undertook to provide treatment intended to save his life.” (Id.)

In *Kruthanooch v. Glendale Adventist Medical Center*, the Court similarly concluded that there was no evidence of neglect when the defendant caused burns to the plaintiff while performing an MRI. There, the Court of Appeals held that “neglect” requires a failure to provide medical care or to attend to the plaintiff’s basic needs and comfort. (83 Cal.App. 5th at 1134-1135.) The Court of Appeals concluded that there was no evidence of neglect because the defendant had harmed the plaintiff “when undertaking medical services.” (Id. at 1135, italics in original.)

Here, as in *Carter* and *Kruthanooch*, there is no allegation that Dr. Diperi withheld care. Indeed, the Complaint contains a litany of things that Dr. Diperi did to provide medical treatment to Plaintiff, although Plaintiff claims that the treatment fell below the standard of care. According to Plaintiff, Dr. Diperi evaluated her, reviewed her diagnostic studies, documented clinical findings and made treatment recommendations. (FAC ¶¶ 27-33, 46-49.) That is the opposite of withholding care. Accordingly, the Court sustains the demurrer to the elder abuse claim against Dr. Diperi with leave to amend.

As to Cedars Sinai, Plaintiff alleges that it “did not provide sufficiently trained staff in sufficient numbers to adequately monitor Ms. Jensen’s condition, timely report to the physicians, and prevent the bowel perforation which was entirely preventable. Defendant Cedars-Sinai Marina Hospital failed to ensure its physicians practicing and accepting patients in Cedars-Sinai Marina Hospital provided adequate care which met the standard of care. Ms. Jensen was dependent on the hospital staff to monitor her and alert the physicians of changes in her condition warranting immediate follow up, yet Ms. Jensen went largely ignored and the staff continued to give her laxatives despite the ineffectiveness and signs and symptoms of a severe impending crisis.” (Id. ¶78.)

Plaintiff’s understaffing allegations are entirely conclusory, and contradicted by more specific allegations in the FAC that she was seen by many doctors at the hospital who run tests and evaluated her (albeit negligently). Plaintiff’s allegations of poor training are also too general. There is no specific allegation as to how the staff was poorly trained, and what they should have reported, why they should have reported it up the chain, and when they should have reported it.

In any event, allegations of understaffing and inadequate training of staff are generally indicative of negligent undertaking of medical services and not elder abuse. (*Worsham v. O’Connor Hospital* (2014) 226 Cal.App.4th 331, 337-338.) While the court in *Fenimore v. Regents of the University of California* (2016) 245 Cal.App.4th 1339, 1349-1350 found that understaffing may support a finding of neglect, it only did so because the plaintiff alleged that the defendant hospital knowingly violated staffing regulations, and had a significant pattern of doing so. (Id. at 1350.) Plaintiff does not allege Cedars had a knowing pattern of violating staffing regulations. Plaintiff does not even allege what the staffing ratio should have been or how more staffing would have prevented her injury.

Plaintiff relies on *Samantha B. v. Aurora Vista Del Mar, LLC* (2022) 77 Cal. App. 5th 85, 97 to argue that “neglect” encompasses more than the withholding of care and can be shown by failing to protect a plaintiff from health and safety

hazards. However, Samantha B. dealt with the failure to protect vulnerable female psychiatric patients from sexual predation. It is not analogous to this case. Samantha B. never held that “health and safety hazards” included an allegedly incompetent doctor who fails to correctly diagnose a patient’s medical condition and provide the care appropriate for that condition.

Plaintiff’s reliance on *Fenimore v. Regents of University of California* (2016) 245 Cal. App. 4th 1339, 1347 is equally misplaced. In *Fenimore*, the patient suffered from dementia, and within minutes of being admitted to defendant hospital, he fell. The hospital covered up his fall from his family and personal physician and doctored medical records to indicate that his fall occurred while nurses were changing his diaper. It did so, in part because Medicare does not cover treatment of a condition that results from the failure to provide proper care. The patient received no treatment or assessment for four days. Later an x-ray showed the patient had a fractured hip. The hospital also had a “pattern and practice of understaffing and undertraining”, to cut costs and increase profits. (Id. at 1343-1345.)

While the Court concluded that the staffing allegations supported a finding of neglect, it also found that the allegations that the hospital (1) allowed the patient to fall within minutes of entering the hospital, (2) failed to timely x-ray the patient’s hip, and (3) delayed the diagnosis and treatment of the fractured hip, amounted to incompetence, not neglect. The Court also noted that “[i]t is not as though the Hospital did nothing at all, even according to the FAC. The nurses inquired about his pain, they gave him acetaminophen for it, and the occupational therapist noted his need for an X-ray.” (Id. at 1348.)

So it is here. Plaintiff’s allegations suggest that Defendants were incompetent in failing to timely diagnose and treat her condition. Under *Fenimore*, those allegations amount to “incompetence or unskillfulness (i.e., negligence)”, not neglect.

Accordingly, the Court sustains the demurrer to Plaintiff’s elder abuse claim.

Vicarious Liability

Cedars argues that the third cause of action for vicarious liability against it is defective because vicarious liability is not an independent cause of action, but a theory of liability. The Court agrees. Vicarious liability is a theory of liability that applies to a tort. It is the underlying tort that creates the cause of action.

Plaintiff argues that she should be given leave to amend so that “[t]he facts alleged in the third cause of action can be incorporated into the malpractice and elder abuse causes of action as agency and apparent-agency allegations against Cedars-Sinai.” But the general allegations in the FAC already allege that Cedars is vicariously liable for the negligence and/or neglect of its doctors. Paragraphs 3-5 of the FAC allege that Cedars is vicariously liable for all the acts and/or omissions of the doctor defendants.

Motion to Strike

Cedars moves to strike Plaintiff’s vicarious liability action because it is not a separate tort. A party cannot move to strike a cause of action. Rather, the proper vehicle to remove a cause of action is through a demurrer. In any event, any such motion to strike is moot given the Court’s ruling above, sustaining the demurrer to the vicarious liability claim.

Cedars also moves to strike Plaintiff’s punitive damages allegations. Cedars makes several arguments for doing so, including that the claim is procedurally barred under Code of Civil Proc. section 425.13(a).

Section 425.13(a) states that “[i]n any action for damages arising out of the professional negligence of a health care provider, no claim for punitive damages shall be included in a complaint or other pleading unless the court enters an order allowing an amended pleading that includes a claim for punitive damages to be filed.” The statute requires that the party seeking leave to amend must file a motion with supporting affidavits showing that he/she has a substantial probability of prevailing on the claim pursuant to Section 3294 of the Civil Code.

Section 425.13(a) does not apply to elder abuse claims. (Covenant Care, Inc. v. Superior Court, (2004) 32 Cal.4th 771, 790.) However, as the demurrer to that claim has been sustained, the only remaining cause of action against Moving Defendants is for malpractice, and the procedural requirements of section 425.13 apply to professional negligence claims. Further, it is undisputed that Plaintiff did not seek leave from the Court to bring a punitive damages claim.

Even if Plaintiff had adequately alleged conduct rising above professional negligence, the FAC still fails to plead the additional facts required to impose punitive damages against a corporate defendant like Cedars. Civil Code section 3294 (b) requires that to seek punitive damages against an entity, Plaintiff must allege that an officer, director, or managing agent personally engaged in, authorized, ratified, or consciously disregarded the allegedly wrongful conduct or had advance knowledge of the employee's unfitness and acted with conscious disregard for the safety of others. (Civ. Code § 3294 (b).) The FAC does not identify any officer, director or managing agent who did any of the above.

Moving Defendants also seek to strike Plaintiff's request for attorneys' fees. In the absence of a statute or contract authorizing attorneys' fees, attorneys' fees are not recoverable. (Code Civ. Proc., section 1021.) The parties agree that there is no contract allowing the recovery for attorneys' fees. Rather, Plaintiff seeks fees as part of her elder abuse claim. As the Court has sustained the demurrer to that claim, the Court will also strike the request for attorneys' fees.

CONCLUSION AND ORDER

Based on the foregoing, the Court SUSTAINS the demurrer with 20 days' leave to amend as to the elder abuse claim and without leave to amend as to the vicarious liability claim. The Court GRANTS the motion to strike the prayer for punitive damages and the request for attorneys' fees with 20 days' leave to amend.

IT IS SO ORDERED.

DATED: September 16, 2026 _____

Edward B. Moreton, Jr.

Judge of the Superior Court